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Prescribing Information

For the use of Registered Medical Practitioner, Hospital or a Laboratory Only.

DICLOTROY EC TABLET 50 mg.

Diclofenac Tablets BP

COMPOSITION:
Each enteric coated tablet contains:
Diclofenac Sodium BP 50 mg.

DESCRIPTION:
Brown colored, round, biconvex, enteric coated tablet.

ACTION AND CLINICAL PHARMACOLOGY:
Diclofenac sodium is a nonsteroidal antiinflammatory drug (NSAID) with analgesic and antipyretic properties. Diclofenac inhibits prostaglandin synthesis by interfering with the action of prostaglandin synthetase.
Although diclofenac sodium does not alter the course of the underlying disease, it has been found to relieve pain, reduce fever, swelling and tenderness, and increase mobility in patients with rheumatic disorders of the types listed.

PHARMACOKINETICS:
Absorption: Diclofenac is rapidly and completely absorbed from the coated tablets. When the tablets are taken with a meal, the quantity of active substance absorbed is not diminished, although the speed of its absorption may possibly be reduced. Distribution Following ingestion of one coated tablet of 50 mg, the plasma concentrations of diclofenac attain a mean peak value of 1.2 µg/mL (3.9 µmol/L) after 20-60 minutes. The plasma concentrations show a linear relationship to the size of the dose. Repeated oral administration in daily doses of 50 mg t.i.d. for a week does not lead to cumulation of diclofenac in the plasma. Diclofenac becomes bound to serum proteins at a rate of 99.7%; chiefly to albumin (99.4%).
Metabolism: Roughly half of the active substance is metabolised during its first passage through the liver (first pass effect); consequently, the areas under the concentration curves (AUCs) after an oral dose are only about one-half as large as after a parenteral dose of the same size. The biotransformation of diclofenac involves partly-glucuronidation of the intact molecule but mainly single and multiple hydroxylation followed by glucuronidation. Approx. 60% of the dose administered is excreted in the urine than 1% as unchanged substance. The remainder of the dose is eliminated as metabolites through the bile in the faeces. No relevant age-dependent differences in the drug's absorption, metabolism, or excretion have been observed.
Elimination: The total systemic clearance of diclofenac in plasma is 263±56 mL/min (x ± SD). The terminal half-life in plasma is 1-2 hours. Kinetics in special clinical situations In patients suffering from renal impairment, no accumulation of the unchanged active substance can be inferred from the singled-ose kinetics when applying the usual dosage schedule. At a creatinine clearance of less than 10 mL/ min, the theoretical steady-state plasma level of metabolites is about four times higher than in normal subjects. However, the metabolites are ultimately cleared through the bile. In the patient of impaired hepatic function (chronic hepatitis, compensated cirrhosis), the kinetics and metabolism of diclofenac are the same as in patients without liver disease.

INDICATION:
The symptomatic treatment of rheumatoid arthritis, ankylosing spondylitis osteo-arthritis and spondylarthritis.

DOSAGE AND ADMINISTRATION:
As a general recommendation, the dose should be individually adjusted. Adverse effects may be minimized by using the lowest effective dose for the shortest duration necessary to control symptoms.
Adults: As a rule, the daily dosage for adults is 100-150 mg. In milder cases, as well as for children over 14 years of age, 75-100 mg daily is usually sufficient. The daily dosage should always be prescribed in 2-3 fractional doses. The tablets should be taken with liquid, preferably before meals.
Children: The dosage strengths are such that diclofenac tablets are not recommended for use in children. After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.
Established cardiovascular disease or significant cardiovascular risk factors: Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤ 100 mg daily if treated for more than 4 weeks (see section Warnings and Precautions).

CONTRA-INDICATIONS:
Patients with severe cardiac failure (see section Warnings and Precautions).
Patients with active, or recent history of, inflammatory diseases of the gastrointestinal tract such as peptic ulcer, gastritis, regional enteritis, or ulcerative colitis.
Known or suspected hypersensitivity to the drug. Since cross sensitivity has been demonstrated, diclofenac sodium should not be given to patients in whom ASA or other nonsteroidal anti-inflammatory agents (NSAIDs) have induced asthma, rhinitis, urticaria or other allergic manifestations. Fatal anaphylactoid reactions have occurred in such individuals.

WARNINGS:

GASTROINTESTINAL EFFECTS
NSAIDs, including diclofenac, may be associated with increased risk of gastrointestinal anastomotic leak. Close medical surveillance and caution are recommended when using diclofenac after gastrointestinal surgery.

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CARDIOVASCULAR EFFECTS
Treatment with NSAIDs including diclofenac, particularly at high dose and in long term, may be associated with an increased risk of serious cardiovascular thrombotic events (including myocardial infarction and stroke).
Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischaemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤ 100 mg daily if treated for more than 4 weeks.
As the cardiovascular risks of diclofenac may increase with dose and duration of exposure, the lowest effective daily dose should be used for the shortest duration possible. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically, especially when treatment continues for more than 4 weeks.
Patients should remain alert for the signs and symptoms of serious arteriothrombotic events (e.g. chest pain, shortness of breath, weakness, slurring of speech), which can occur without warnings. Patients should be instructed to see a physician immediately in case of such an event.

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID
Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAIDs therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms. Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events. Cardiovascular Thrombotic Events Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration. There is no consistent evidence that the concurrent use of aspirins mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use. Hypertension NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter. Heart Failure Fluid retention and oedema have been observed in

some patients taking NSAIDs, there caution is advised in patients with fluid retention or heart failure. Gastrointestinal Events All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patient about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increased the risk of serious gastrointestinal adverse events. Severe Skin Reactions NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

PRECAUTION:
Severe cutaneous reactions, including Stevens - Johnson syndrome and toxic epidermal necrolysis (Lyell's syndrome), have been reported with diclofenac sodium. Patients treated with diclofenac sodium should be closely monitored for signs of hypersensitivity reactions. Discontinue diclofenac sodium immediately if rash occurs.

Pregnancy and Lactation:
The safety in pregnancy and lactation has not been established and its use is therefore not recommended. It should only be used during pregnancy for the most compelling reasons, and then only at the lowest effective dose.

DRUG INTERACTIONS:
ASA: Serum levels of diclofenac may be reduced when the two drugs are taken simultaneously. The bioavailability of ASA is reduced by the presence of diclofenac.
Digoxin: Diclofenac may increase the plasma concentration of digoxin. Dosage adjustment may be required.
Lithium: Lithium plasma concentrations will increase when administered concomitantly with diclofenac (which affects lithium renal clearance). Dosage adjustment of lithium may be required.
Oral hypoglycemic drugs: Pharmacodynamic studies have shown no potentiation of effect with concurrent administration with diclofenac; however, there are isolated reports of both hypoglycemic and hyperglycemic effects in the presence of diclofenac, which necessitated changes in the dosage of hypoglycemic agents.
Anticoagulants: Although clinical investigations would appear to indicate that diclofenac has no influence on the effect of anticoagulants, there have been isolated reports of an increased risk of hemorrhage with the combined use of diclofenac and acenocoumarol anticoagulant therapy. Special caution is therefore recommended and frequent laboratory tests should be performed to check that the desired response to the anticoagulant is being maintained.
Diuretics: NSAIDs have been reported to decrease the activity of diuretics. Concomitant treatment with potassium-sparing diuretics may be associated with increased serum potassium, thus making it necessary to monitor levels.
Glucocorticoids: Concomitant administration may aggravate gastrointestinal side effects.
NSAIDs: Concurrent oral treatment with two or more NSAIDs may promote the occurrence of side effects.
Methotrexate: Caution should be exercised when NSAIDs are administered less than 24 hours before or after treatment with methotrexate. Elevated blood concentrations of methotrexate may occur, increasing toxicity.
Cyclosporine: Nephrotoxicity of cyclosporine may be increased because of the effect of NSAIDs on renal prostaglandins.
Quinolone antibacterials: There have been isolated reports of convulsions which may have been due to concomitant use of quinolones and NSAIDs.
Antihypertensive agents: Like other NSAIDs, diclofenac can reduce the antihypertensive effects of propranolol and other B-blockers, as well as other antihypertensive agents.
Clinical Laboratory Tests: Diclofenac increases platelet aggregation time but does not affect bleeding time, plasma thrombin clotting time, plasma fibrinogen, or factors V and VII to XII. Statistically significant changes in prothrombin and partial thromboplastin times have been reported in normal volunteers. The mean changes were observed to be less than 1 second in both instances, and are unlikely to be clinically important.
Persistently abnormal or worsening renal, hepatic or hematological test values should be followed up carefully since they may be related to therapy.

ADVERSE REACTIONS:
Gastrointestinal, dermatological and CNS adverse reactions are most commonly seen. The most severe gastrointestinal adverse reactions observed were ulceration and bleeding while the most severe dermatological albeit rare reactions observed were erythema multiforme (Stevens-Johnson syndrome and Lyell's syndrome). Fatalities have occurred on occasion, particularly in the elderly.
Gastrointestinal: Occasional: epigastric, gastric, or abdominal pain, abdominal cramps, nausea, dyspepsia, anorexia, diarrhea, vomiting, flatulence.
CNS: Occasional: dizziness, headache, vertigo.
Special Senses: Isolated: vision disturbances (blurred vision, diplopia), impaired hearing, tinnitus, taste alteration disorders.
Cardiovascular:
Cardiac disorders:
Kounis syndrome: Frequency "not known"
Uncommon: Myocardial infarction, cardiac failure, palpitations, chest pain.
(The frequency reflects data from long-term treatment with a high dose 150mg/day)
Arteriothrombotic events: Meta-analysis and pharmacoepidemiological data point towards an increased risk of arteriothrombotic events (for example myocardial infarction) associated with the use of diclofenac, particularly at a high dose (150mg daily) and during long-term treatment (see section Warnings and Precautions).
Rare: arrhythmias, hypertension.
Dermatological: Occasional - rashes or skin eruptions Cases of hair loss, bullous eruptions, erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis (Lyell's syndrome), and photosensitivity reactions have been reported.
Renal System: Rare: edema (facial, general, peripheral). Isolated: acute renal failure, nephrotic syndrome.
Hematologic: Isolated: thrombocytopenia, leukopenia, agranulocytosis, hemolytic anemia, aplastic anemia.
Hepatic: Occasional: elevations (^{3}3 times the upper normal limit) of serum aminotransferase enzymes (AST, ALT).
Hypersensitivity:
Rare: hypersensitivity reactions such as asthma in patients sensitive to ASA e.g., bronchospasm; anaphylactic / anaphylactoid systemic reactions including hypotension.
Isolated: vasculitis, pneumonitis.

SYMPTOMS AND TREATMENT OF OVERDOSE:
Symptoms and Treatment: There is no specific antidote. In cases of overdosage, absorption should be prevented as soon as possible by the induction of vomiting, gastric lavage or treatment with activated charcoal. Supportive and symptomatic treatment should be given for complications such as hypotension, renal failure, convulsions, gastrointestinal irritation and respiratory depression. Measures to accelerate elimination (forced diuresis, hemoperfusion, dialysis) may be considered, but may be of limited use because of the high protein-binding and extensive metabolism.

STORAGE:
Store below 30°C, protected from light.

PRESENTATION:
Available in pack of 10 tablets.

DATE OF REVISION:
12/05/2026

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